**Global status report on physical activity 2022**

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## The purpose of this report

This first *Global status report on physical activity* presents a synthesis of global progress on implementation of GAPPA recommendations. The report was requested by Member States under World Health Assembly Resolution 71.6 and describes the current situation – and where possible, recent trends – using the best available data and set of 29 indicators. Data for this report are drawn from two primary sources: the WHO Noncommunicable Disease Country Capacity Survey (2019 and 2022) and the *WHO Global status report on road safety* (2018). These data also offer the first insight into the impact of COVID-19 on countries' capacity for, and progress towards, implementing policies related to physical activity.

This report is for everyone involved in promoting and delivering national and subnational policies that provide the environments and programmes that drive participation in physical activity. It provides five recommended actions to promote physical activity as the world continues to respond to, and recover from, the COVID-19 pandemic, and to accelerate action to achieve the SDGs – particularly SDG 3 on health and well-being.

## Main findings

There are few areas in public health – such as physical activity - where evidence on required action is so convincing, cost effective and practical. While some countries have started to implement different recommended GAPPA policy actions, overall global implementation since its adoption 5 years ago has been **slow and uneven,** resulting in little progress towards increasing population levels of physical activity. A consequence of this "inaction" is that already stretched health systems are burdened with preventable disease today and even more so in the future, and communities fail to benefit from the wider social, environmental and economic benefits associated with more people being more active.

Only two GAPPA policy indicators show implementation by over three quarters of all countries: conducting national surveillance of physical activity (among adults, and among children and adolescents); and the presence of national road safety design standards for safe crossings for pedestrians and cyclists. For nine GAPPA policy indicators, between a half to two thirds of countries report implementation. For the remaining 18 GAPPA policy indicators, less than half of countries report implementation in 2021 (see Table 1). Uneven implementation across WHO regions and country-income level results in **inequities** in people's access to opportunities and environments that support being regularly and safely active.

This first global assessment of policy actions to increase physical activity reveals **significant gaps in policy implementation**. These gaps are demonstrated by the overall modest level of GAPPA-recommended policy development and the notable gap between the presence of a policy and its operational status. In the majority of countries, policy development and implementation must be strengthened and accelerated if global targets on physical activity are to be met.

Gaps in policy are a result of multiple interconnected factors that fall into five areas: **political, technical, financial, collaboration and capacity building, and data systems** – all of which can either limit or accelerate policy progress. When positively aligned, these "policy enabling" factors combine to set and advance the national agenda. Conversely, the absence of one or more of these factors can reduce, divert and even reverse policy progress. As the national policy cycle for physical activity is not "one size fits all", all countries need to identify and strengthen the policy enablers to drive a positive and virtuous cycle of collective action to enable more physical activity.

## GAPPA policy implementation: results by policy area

### GAPPA policy area – active systems:

The number of countries reporting a national NCD policy (including physical activity, or a standalone physical activity policy) has increased since 2017 and 2019. However, this progress is tempered by the 28% of countries that report these policies are not being implemented in 2021 (i.e. reported as not "operational"). These results, combined with evidence that less than 50% of countries report having a national NCD coordinating mechanism to support multisectoral collaboration, are of major concern and contribute to explaining the low level of policy implementation on physical activity seen across the indicators presented in this report.

### GAPPA policy area – active societies:

Just over half of countries report conducting at least one communication campaign to raise awareness and knowledge around physical activity in the past two years – a figure that has declined since 2019. About half of countries implemented mass-participation events to engage people in physical activity through free, community-wide events. This figure has also declined since 2019, likely due to the COVID-19 pandemic.

### GAPPA policy area – active environments:

Global progress in policy action to provide environments that support physical activity is varied. National design standards for road safety features that protect people when walking and cycling are present in three quarters (76%) of countries for safe road crossings, and two thirds (66%) of countries for design for the safe management of speed. Only half of countries report national standards requiring separated infrastructure for walking and cycling, and less than half report the presence of all three of these national road safety design standards. Furthermore, while legislation on speed limits and drink–driving is present in most countries, only a quarter (26%) of these countries' legislation meets WHO best-practice standards. Low levels of best-practice legislation combined with an absence of road design standards presents increased risks to people walking and cycling in these local communities.

### GAPPA policy area – active people:

Implementation of policies that ensure opportunities for physical activity in key settings where people live, work and play, and targeted programmes to support key population groups, is reported by less than half of all countries. Notably, less than 40% of countries report having national protocols for the management of physical activity in primary health care, despite strong evidence of the protective benefits of regular physical activity against leading NCDs and for mental health. Although this indicator shows a modest increase since 2019, the slow level of implementation of this recognized "best buy" policy is of particular concern given the high number of people living with, or at risk of, NCDs.

## Strengthening data for global and national GAPPA monitoring

Monitoring GAPPA implementation, outcomes, and impact will help evidence-based decision-making and country and global progress. This report presents results based on the 29 policy implementation indicators identified in the GAPPA monitoring framework. While global data exist to monitor the presence of national policies, strategies and action plans covering NCDs, physical activity, walking and cycling, public transport and road safety, there are clear data gaps. These include a lack of detail such as on the policy content, and on policy reach and enforcement.

At least two improvements are needed for future GAPPA monitoring. First, there is a need to develop additional indicators to ensure full coverage of all GAPPA policy recommendations. Second, there is a need to review and strengthen existing data collected for those indicators currently identified. To address some of the identified gaps, new data collection systems and instruments may be needed. It is possible that conducting a periodic global survey to assess national progress on GAPPA implementation may be required. This approach is used for other NCD risk factors (for example tobacco use and unhealthy diet) and the applicability and feasibility to do the same for physical activity should be explored.

Gaps were also identified in current global and national data systems to track levels of physical activity over time, across the life-course, and in key subpopulations. These include the absence of national surveillance of physical activity among people living with disability, as well as among children aged 6–9 years and children under the age of 5 years. In addition, there are no global data on sedentary behaviours and key domains of physical activity such as sport and active transport (walking and cycling). These deficiencies in global and national surveillance systems must be addressed. Multiple government departments can benefit from coherent and consistent data on trends in specific physical activity domains, and such data must be collected and harmonized to maximize use and efficiency.

Given the potential advantages of wearable and mobile devices to support tracking of physical activity, global consensus is needed on the technical tools and protocols for (as well as action to mitigate the financial barriers to) their use in national and global monitoring systems. Regular updates on global levels of physical activity are needed to report on progress towards the primary GAPPA outcome of increasing physical activity by 15% by 2030.

As GAPPA called for a multisectoral, whole-of-system policy response, monitoring GAPPA implementation and impact also requires a whole-of-system approach. There is an urgent need to strengthen the methods and tools to capture the total health, social, and economic costs and returns of increasing physical activity. These data are vital to ensure and reinforce the engagement of all relevant sectors, not just sport and health, and include strengthening the use of impact assessment tools, particularly in transport and urban planning decision-making.

## The way forward: five recommendations

Accelerating physical activity policy implementation is vital to achieve targets set for 2030. It will require identifying opportunities across government and mitigating policy constraints within each country. To assist countries, five actions aimed at closing the policy-implementation gap are recommended.

1. **Strengthen whole-of-government ownership and political leadership**

- Establish and reinforce leadership on physical activity within and across all relevant government departments and at all levels.
- Invest in advocacy for policy action on physical activity.
- Increase awareness of the crosscutting contributions of physical activity policy to national development and to multiple global priorities as set out in the SDGs.

## 1.3 Aim of this report

The *Global status report on physical activity* presents the first dedicated synthesis of global progress on implementation of the recommended policy actions outlined in GAPPA across four strategic policy areas, namely active societies, active environments, active people and active systems. The report was requested by Member States under World Health Assembly Resolution 71.6 *(1)*, and presents a picture of the current situation and (where possible) recent trends using the best available data and an existing set of agreed global indicators.

As we approach the halfway mark for achieving Sustainable Development Goal (SDG) 3 on good health and well-being, including target 3.4 on reducing premature mortality resulting from NCDs, this report serves as a clarion call and advocacy tool to accelerate political action. It also serves as a resource to guide policy implementation by government actors, including ministries of health, sport, education, transport and finance; local authorities; and all other relevant stakeholders. The objectives of the report are to:

- summarize the health benefits of physical activity, current levels of physical activity, and current global recommendations;
- provide new global estimates on the economic cost of physical inactivity to health systems;
- present an assessment of current GAPPA policy implementation;
- highlight areas of progress and identify gaps in GAPPA implementation;
- identify data needs to strengthen future reporting on global progress on physical activity; and
- present five recommendations on how countries can accelerate progress on policy implementation to increase physical activity.

## 1.4 Who this report is for

Effective action to promote physical activity in all its forms across multiple settings requires all key sectors to work in partnership in order to strengthen their collective impact. This report is intended for all stakeholders involved in promoting and delivering national and subnational policies that provide the environments and programmes that drive physical activity participation. This includes:

- policy-makers and officials across national and subnational government sectors, including health, welfare, education, transport, urban planning, sport, tourism, and finance;
- NGOs, civil society organizations (CSOs) and social enterprises;
- the private sector and employers;
- sport and exercise organizations;
- academics and researchers;
- teachers and training institutions; and
- public health advocates on NCD prevention, mental health, healthy ageing, road safety, and health promotion.

These key advocates and decision-makers may work at global, regional, national, or community level to integrate action on physical activity into stronger health systems, as well as COVID-19 recovery plans, environmental and urban health mitigation agendas and national development plans.

# 2.2 The economic burden of physical inactivity

Physical inactivity is associated with a range of chronic diseases and premature deaths *(2)* that carry not just a human but also an economic cost. Estimating the economic burden of physical inactivity is important to inform policy and decide resource prioritization. Previous global estimates of the economic burden of physical inactivity, undertaken in 2016, showed a global cost in 2013 of INT$ 67.5 billion per year due to health care expenditure and productivity losses *(9)*.

### 2.2.1 Methods for estimating the burden

Using standard methods *(9, 10)* the directly attributable health care costs of physical inactivity to the health system were estimated using the best data available for 142 countries, representing 93.2% of the world's population. Direct costs were estimated for the period 2020–2030 for coronary heart disease, stroke, type-2 diabetes, hypertension, seven cancers (breast, colon, endometrial, gastric, oesophageal, renal, and bladder), and, for the first time, two mental health outcomes, namely dementia and depression *(11)*.

National estimates of the prevalence of physical inactivity from countries in 2016 *(6)* were used to calculate population-attributable fractions (PAFs) associated with physical inactivity for each disease outcome. More detailed description of the methods are provided in Annex 1 *(11)*.

### 2.2.2 The cost of physical inactivity

The economic burden of physical inactivity is large. Globally, almost 500 million (499 208 million) new cases of preventable NCDs will occur between 2020–2030, incurring treatment costs of just over US$ 300 billion (INT$ 524 billion) or around US$ 27 billion (INT$ 48 billion) annually if there is no change in the current prevalence of physical inactivity.

The burden of new cases will largely fall on lower- and upper-middle-income countries, which are set to account for nearly three quarters – 74% – of estimated new cases of NCDs, with the Western Pacific Region predicted to be hardest hit (see Fig. 5) *(11)*.

Globally, nearly half of new NCD cases (47%) will result from hypertension, and 43% will result from depression. The new cases of these two diseases will account for 22% and 28% respectively of total direct health care costs. Meanwhile, 21% of costs will be incurred for the treatment of dementia, even though this only accounts for 3% of total preventable cases (see Fig. 6). This high cost is due to the nature of dementia management treatment, and the duration for which it is needed.

As might be expected, and similar to findings in 2016 *(9)*, the economic burden of physical inactivity is unequally distributed across regions and is disproportionate in relation to the disease burden. The largest economic cost is set to occur among high-income countries, which will account for 70% of expenditure on treatment for illness resulting from physical inactivity.

These estimates show that society is paying the price for not acting to reduce levels of physical inactivity. Moreover, these estimates are conservative. The inclusion of other important health outcomes, for example costs of treatment and management of preventable cases of falls and their associated injuries, would provide a comprehensive estimate of the impact of physical inactivity on health systems. In addition, if models were extended to include productivity losses due to morbidity and mortality, they would reveal the even greater economic costs associated with physical inactivity *(12)*.